

Launch Progress Report

Startup Name · Cohort & Program Year · Reporting Period

Startup Profile

Startup Name:	ClinicalHours
Founders:	Shivam Kanodia/ Raghavsai Tirupati
Industry:	Healthcare Technology/Clinical Operations
Stage:	Pre-seed
Cohort:	Cohort 1: 2026
Report Period:	March 29, 2026 – April 26, 2026
Website:	clinicalhours.org

1. Progress Made This Period

Milestone / Goal	Target Date	Status	Notes
MVP Launch	01/29/2026	✓ Complete	Accomplished this in January
100 Student Users	02/25/2026	✓ Complete	Accomplished this in February on the student side
First Clinic Pilot	04/08/2026	✓ Complete	Working since mid-march, official on April 8th
First paying clinic	06/01/2026	In progress	We're working towards onboarding more clinics

Milestone Tracker

Key Wins & Highlights

- We partnered with a clinic in early April (BCS Free Health Clinic) and are running an unpaid pilot program through them, iterating on our product while expanding our network in healthcare.
- We've pivoted from a student-side focus to an enterprise focus, marketing ourselves as a company that provides innovative volunteering infrastructure for clinics. However, we are still marketing to students to expand our network on both sides to build our moat in this industry.
- We placed third at Good Bull Pitch, were Finalists for the Ideas Challenge, and won third in the Meloy Bullet Pitch.

2. Future Plans & Next Steps

30-Day Priorities

- Primary Goal: Clinic deployment + user acquisition
- Get BCS FHC to activate our platform by May 15 (waitlist + intake flow live)
- Drive 100+ student signups through BCS FHC referral
- Prepare a case study template for future clinic sales
- Secondary Goal: Expand clinic pipeline
- Pitch 5 additional clinics (UTA Health Center, Dallas small clinics, Baylor Connection)
- Aim for 2 paid pilots by the end of May
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- Slow-moving healthcare systems, including the BCS FHC, are slowing our growth. However, we counteract this by proactively pursuing more pilots/partnerships with other clinics.

60–90 Day Goals

- **[What does success look like two to three months from now?]**
- 5 clinic partnerships (majority paying)
- 500+ total student users
- Expanded into UNT, UTA, UTD pre-med orgs (via clinic referrals, not direct student outreach)
- **[Key product, growth, or fundraising milestone.]**
- Fundraising/growth: \$500–2k/month from 2–3 clinic partners (unpaid pilots converting to paid)
- **[Hires or partnerships you are actively pursuing.]**
- Baylor Scott & White: referral program for rejected applicants
- Small Dallas clinics: direct partnerships
- Mentors: Mark Radolovich, Ben Wiggins (clinical ops expertise)

Support Needed from Accelerator

- **Mentors** in clinical operations, Mark Radolovich and Ben Wiggins, have been invaluable. We'd benefit from 2–3 more mentors with clinic leadership experience (e.g., volunteer coordination, clinic operations, HR).
- **Legal guidance:** Healthcare compliance (volunteer liability, data privacy). Our on-campus resources can handle basic contracts, but clinical ops require specialized review.
- **Clinic warm intros:** Any connections to clinic leadership in DFW or Central Texas (especially safety-net clinics, hospital volunteer pr

3. Lessons Learned

What Worked Well

- Lean go-to-market: Idea → MVP → first pilot in 2.5 months
- Founder-led sales: Direct conversations with clinic leaders shaped the product roadmap
- Pivot speed: Saw market signal (students = low value) → shifted focus (clinics = real pain)
- Two-sided network: Once clinics use us, students naturally follow
- We are going to double down on our emphasis on clinical partnerships. This was a pivot in the past, and I was unsure of it. However, I have seen advice from judges in pitch comps and from my own outreach that this is where the market truly is.

What Did Not Work

- **Student-first marketplace:** The assumption that university status = acquisition advantage was wrong
- **AI feature wrapping:** "AI resume builder" and similar features were valueless to students without underlying clinic partnerships
- **Low-touch growth:** Clinics require personal outreach, not viral loops

Pivots or Changes Made

We started with a flawed assumption: pre-med students would use a centralized database to find clinical opportunities. We quickly learned:

- Market was too small (fragmented by region, institution)
- No payment willingness from students
- Clinics were the actual pain point

Why we pivoted to clinics:

We spoke with William Shaw (volunteer manager, St. Joseph Health). He showed us the gap: clinics manage volunteers through spreadsheets and outdated tools. Competitors focus on scheduling logistics rather than volunteer quality or integration with clinic workflows.

Our differentiation: We're building volunteer operations infrastructure designed for clinics that want to scale their volunteer programs. We position it as an all-in-one application + management tool.

Advice for Other Founders

Launch as fast as possible, pivot as needed, and then GET USERS

4. Funding Request & Billable Sheet

List all items for which you are requesting accelerator funding below. Attach receipts or quotes as supporting documents where applicable.

#	Item / Service	Category	Qty	Unit Cost (\$)	Total (\$)	Justification
1	LLC Formation	Legal	1	\$348	\$348	Formal Entity + Notary
2	Apple Developer Account	Business Growth	1	\$99	\$99	App Store Development + Launch
3	API Cost	Product	1	\$178	\$178	See API Usage Section
4	Clinic Marketing + Student Ambassadors	Business Growth	1	\$850	\$600	Ambassador stipends (6–10 UTA/UNT/UTD pre-health orgs) at \$75–100/semester
5	[Description]	API Costs	1	\$0.00	\$0.00	[Reason for expense]
TOTAL REQUESTED					\$1,175	

Legal: \$348

- Texas LLC formation via Secretary of State: \$308
- Notarized founders' agreement (equity/IP assignment): \$40
- Will use on-campus law resources (UT law school advisors, AggieX) for clinic contracts

Apple Developer: \$99

- Students access volunteer opportunities via an iOS app
- Clinics requested mobile access; this unblocks that requirement

Growth: \$600

- Student Ambassador Program (NOT influencer/TikTok): 6–10 pre-health orgs across UTA, UNT, UTD
- Each ambassador: \$75–100/semester + revenue share on premium student features (if launched later)

Why: Clinic referrals are primary. Ambassadors = credibility with student networks for those clinics

API Costs: \$128

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- Anthropic Haiku 4.5 for volunteer matching engine (~\$0.01-0.05 per call)
 - Cost estimate: \$1.30/day × 90 days = \$117 (includes overhead)
 - Plus Gemini API for marketing assets: ~\$11
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